

The TRT Readiness Guide

BabyBird Physique | Wesnoski Coaching

Why Most Men Aren't Ready for TRT

Low energy, stubborn belly fat, no drive in the gym, poor recovery. These point toward hormones, but hormones don't work in isolation. They sit downstream of your gut, liver, inflammation and nutrient status.

Start TRT without fixing that foundation and you can amplify the same problems. This guide covers what labs to run, what your GI-MAP shows, and what to address before or alongside any hormone therapy.

How to use this guide

Run your labs first. Then review your GI-MAP. Then build your protocol from the support sections with your provider. The goal isn't to avoid TRT. It's to make sure your body is ready to respond to it.

The Lab Panel

Ranges vary by lab, so always compare against your own report. "Optimal" ranges reflect common functional-medicine targets, not lab standards.

Hormone Markers

Marker	Standard Range	Optimal Range	Why It Matters
Total Testosterone	~264-916 ng/dL	700-1,100 ng/dL	Energy, muscle, drive. Low T is confirmed with two early-morning tests on different days, not one.
Free Testosterone	Varies by test method	Upper half of lab range	The usable portion. Often more telling than total.
Estradiol (sensitive)	~8-43 pg/mL	10-30 pg/mL	Too high or too low both hurt libido, mood and body composition.
SHBG	~10-56 nmol/L	20-40 nmol/L	Binds testosterone. High SHBG = less free T.
LH & FSH	Varies by lab	LH: 3-8 IU/L; FSH: 2-8 IU/L	Shows whether the issue starts in the testes or the brain.
Prolactin	~2-18 ng/mL	5-15 ng/mL	High prolactin suppresses testosterone.
DHEA-S	Depends on age	400-600 µg/dL	Adrenal hormone. Often low with long-term stress.

Metabolic & Thyroid Markers

Marker	Standard Range	Optimal Range	Why It Matters
Fasting Insulin	~2.6-24.9 uIU/mL	2-6 µIU/mL	Insulin resistance drives fat gain and more testosterone-to-estrogen conversion.
Fasting Glucose	~70-99 mg/dL	75-85 mg/dL	Poor blood sugar control is linked to lower T.
HbA1c	Under 5.7%	4.8-5.3%	3-month blood sugar average.
TSH	~0.45-4.5 mIU/L	1.0-2.0 mIU/L	Low thyroid mimics low-T symptoms.
Free T3	~2.0-4.4 pg/mL	3.4-4.2 pg/mL	Active thyroid hormone. Energy and metabolism.
Free T4	~0.8-1.8 ng/dL	1.3-1.7 ng/dL	Storage form. Conversion problems are common.
Reverse T3	~9-24 ng/dL	Under 12 ng/dL	Rises with stress, long diets and inflammation.

Inflammation & Liver Markers

Marker	Standard Range	Optimal Range	Why It Matters
hs-CRP	Under 3.0 mg/L	Under 1.0 mg/L	Whole-body inflammation.
Homocysteine	Under ~15 µmol/L	6-8 µmol/L	Methylation marker. High = B vitamin and inflammation flag.
AST / ALT	~10-40 / ~7-56 U/L (varies)	15-25 U/L	Liver enzymes. Also rise after hard training, so retest after 3-5 days off.
GGT	~0-60 U/L (varies)	10-25 U/L	Liver-specific stress marker. Not raised by training.
ALP	~44-147 U/L	60-100 U/L	Bile duct and liver health.
Ferritin	~30-400 ng/mL	100-150 ng/mL	Iron stores. Also rises with inflammation.

Nutrient Markers

Marker	Standard Range	Optimal Range	Why It Matters
Vitamin D (25-OH)	30-100 ng/mL	40-50 ng/mL	Low vitamin D is linked to lower testosterone.
Magnesium (RBC)	~3.7-7.0 mg/dL (varies)	5.5-6.5 mg/dL	Sleep, stress and hundreds of enzyme reactions.
Zinc (plasma)	~60-130 mcg/dL	80-110 mcg/dL	Zinc deficiency lowers testosterone.
Vitamin B12	~200-1,250 pg/mL	600-1,000 pg/mL	Methylation, energy, nerves.
Folate (RBC)	Over ~280-500 ng/mL (varies)	Over 500 ng/mL	Methylation support.
Iron panel / TIBC	Varies	Serum iron: 85-130 µg/dL; TIBC: 250-350 µg/dL	Low iron causes fatigue that looks like low T.

TRT Safety Baseline

Get these before starting so your provider can track them once you're on TRT.

Marker	Why It Matters
CBC / Hematocrit	TRT raises red blood cells. A baseline lets you catch thick blood early.
PSA (men 40+)	Required before TRT by urology guidelines.

The GI-MAP

The GI-MAP is a DNA-based stool test that shows your gut bacteria, gut lining health and how well your gut is clearing hormones. Run it before any TRT decision.

The Estrobolome

The estrobolome is the group of gut bacteria that handle estrogen. When it's off, estrogen your liver tried to get rid of can come right back:

Markers That Affect Hormones

Marker	What It Means for Hormones
Beta-glucuronidase	A key hormone marker. High levels unpack estrogen in the gut so it's reabsorbed. Driven by dysbiosis, low fiber, low plant variety and constipation.
Secretory IgA	Your gut's front-line immune defense. Low levels often show up with chronic stress.
Calprotectin	Gut inflammation. Hurts absorption of zinc, magnesium and B vitamins.
Elastase	Pancreatic enzyme output. Low = poor protein and fat digestion.
Steatocrit	Fat in stool. High = poor fat absorption, including vitamins A, D, E and K.
Zonulin (add-on)	A leaky gut marker. Useful, but stool zonulin results should be read with caution.

Bacteria to watch

H. pylori: can lower stomach acid and impair B12 absorption

Klebsiella and Citrobacter: can add to beta-glucuronidase and inflammation

Bacteroides fragilis: linked to gut lining damage

Candida / yeast: a dysbiosis driver

The Foundation Protocol

Build this with your provider based on your labs and GI-MAP. You don't need everything here. Start with what your results point to.

Gut Lining

Supplement	Dose	Why
L-Glutamine	5-10 g daily	Main fuel for gut lining cells. Empty stomach; build up slowly.
Zinc Carnosine	75 mg daily	Protects and repairs the gut lining. Contains about 16 mg zinc, which counts toward your daily zinc.
Colostrum (bovine)	500 mg-2 g daily	Growth factors that support gut repair.
DGL Licorice	400-800 mg before meals	Coats and protects the lining. Use DGL, not whole licorice.
Slippery Elm / Marshmallow	400 mg-1 g, up to 3x daily	Soothing coating for the gut lining.
Aloe (inner leaf)	50-200 mg daily	Calms the lining. Use decolorized, anthraquinone-free.
Butyrate	300-600 mg daily	Fuel for colon cells. Tightens the gut barrier.

Estrogen Clearance

Supplement	Dose	Why
DIM	200-400 mg daily with food	Shifts estrogen toward safer pathways. Can drop estradiol too low, especially on TRT, so recheck E2.
Calcium D-Glucarate	500-1,500 mg daily, split	Blocks beta-glucuronidase so packaged estrogen leaves the body. Best fit for high beta-glucuronidase.

Inflammation

Supplement	Dose	Why
Omega-3 (EPA/DHA)	2-4 g daily with meals	Lowers inflammatory signaling.
Curcumin	500-1,000 mg daily	Broad anti-inflammatory. Use an absorbable form (Meriva, BCM-95).
Boswellia (AKBA)	100-400 mg daily	Calms inflammatory pathways. Look for 30%+ AKBA.
Quercetin	500-1,000 mg daily	Calms mast cells and histamine.
Resveratrol	250-500 mg daily	Antioxidant. Aromatase effects are mostly shown in lab studies.
NAC	600-1,200 mg daily	Builds glutathione. Lowers oxidative stress.
Magnesium Glycinate	300-400 mg nightly	Supports sleep, stress and inflammation.
Vitamin D3 + K2	Dose to your labs	Upper limit is 4,000 IU/day without testing. Higher only with retesting. Pair with K2 (MK-7).
Zinc	15-30 mg daily	Needed to make testosterone. Keep total zinc (including zinc carnosine) at or under 40 mg/day; add 1-2 mg copper.

Liver & Bile Flow

Your liver processes estrogen, and bile carries it out through the gut. Slow bile flow means estrogen backs up.

Supplement	Dose	Why
Milk Thistle	400-600 mg daily	Protects liver cells.
TUDCA	250-500 mg daily	Protects the liver and improves bile flow.
Phosphatidylcholine	1-3 g daily	Main component of bile. Keeps bile thin.
Choline (CDP or Alpha-GPC)	300-600 mg daily	Needed for bile. Low choline is linked to fatty liver.
Taurine	1-3 g daily	Helps bile acids work.

Supplement	Dose	Why
Artichoke / Dandelion	300 mg-1 g daily	Gentle bile stimulants.
Betaine	500 mg-1 g daily	Supports methylation and liver fat metabolism.
Glutathione (liposomal)	250-500 mg daily	Main detox antioxidant.
Alpha Lipoic Acid	300-600 mg daily	Recycles glutathione.
Methylated B Complex	1 cap daily	Cofactors for liver detox and methylation.
Digestive support	With meals, as needed	Low elastase: pancreatic enzymes. High steatocrit or poor fat digestion: ox bile.

Stress & Cortisol

Chronic stress and high cortisol are linked to lower testosterone, worse sleep and more inflammation.

Supplement	Dose	Why
Ashwagandha (KSM-66)	300-600 mg daily	Lowers cortisol. Small trials show a testosterone boost. Rare liver injury reports, so stop if you see yellow skin or dark urine.
Rhodiola	200-400 mg daily	Improves stress response.
Phosphatidylserine	400-800 mg daily	Blunts the cortisol spike from training.
Vitamin C	1-2 g daily	Supports the adrenals and post-training cortisol.
Licorice Root	250-500 mg, short term	Only if cortisol is tested low. Raises blood pressure.

A note on peptides

BPC-157 and KPV are popular for gut repair, but neither is FDA-approved and most evidence is from animal studies. An FDA advisory panel voted in July 2026 to recommend allowing BPC-157 in compounding. The FDA hasn't made a final decision. BPC-157 is banned by WADA, which matters if you compete in tested sports. Only use peptides through a licensed provider.

Lifestyle Foundations

Sleep

Most of your daily testosterone is released during sleep. In one study, young men limited to 5 hours a night for a week saw testosterone drop 10-15%, about the same as aging 10-15 years.

Aim for 7+ hours

Magnesium glycinate 300-400 mg, apigenin 50 mg, L-theanine 200 mg, melatonin 0.5-1 mg

Get tested for sleep apnea if you snore or wake up tired. It's a common hidden cause of low T.

Diet

Eat a wide range of colorful vegetables and fiber. Fiber helps carry estrogen out

Cut back on fried and ultra-processed foods

Prioritize wild-caught fish

Manage blood sugar

Limit alcohol. It raises estrogen and burdens the liver

Bone broth, collagen and fermented foods support the gut lining

Xenoestrogens

These are chemicals that act like estrogen in the body. Cut your exposure where you can:

Use glass or stainless steel instead of plastic

Filter your drinking water

Avoid canned foods and non-stick pans

Swap fragranced products for fragrance-free and paraben-free ones

Body Fat

Fat tissue converts testosterone into estrogen. Losing body fat is one of the most effective ways to improve your ratio.

TRT Readiness Checklist

The more you can check off, the better your results on TRT are likely to be.

Labs done

☐ Total T (2 morning tests) and free T

☐ Sensitive estradiol, SHBG, LH, FSH, prolactin

☐ Full thyroid panel

☐ Fasting insulin, glucose, HbA1c

☐ Liver panel, hs-CRP, homocysteine

☐ Vitamin D, RBC magnesium, zinc, B12

☐ CBC/hematocrit and PSA (40+)

GI-MAP reviewed

☐ Beta-glucuronidase

☐ Dysbiotic bacteria

☐ Elastase and steatocrit

☐ Secretory IgA and calprotectin

Foundation in place

☐ Gut, liver and inflammation support based on your results

☐ Sleep 7+ hours, apnea ruled out

☐ Stress managed

☐ Xenoestrogens reduced

☐ Fertility plans discussed

A discussion checklist for your provider, not a medical clearance or readiness score.

Fertility comes first

TRT shuts down sperm production. If you want kids now or in the future, talk to your provider before starting. Fertility-preserving options exist.

The Bottom Line

TRT can be life-changing, but it's a tool, not a foundation. The men who get the best results do the work first: clear estrogen, heal the gut, lower inflammation and support the liver. Run the labs. Read the GI-MAP. Build the protocol. Then make your decision.

This guide is for educational purposes only and is not medical advice. Don't start, stop or change any medication or supplement without talking to your physician.

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